

Melanotan I

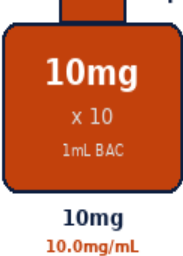
alpha-MSH Analogue | 10mg x 10 Vials

Melanogenesis Stimulation | UV-Independent Tanning | SubQ Injection

Purity: 99.8% | Endotoxin-Free | Research Use Only

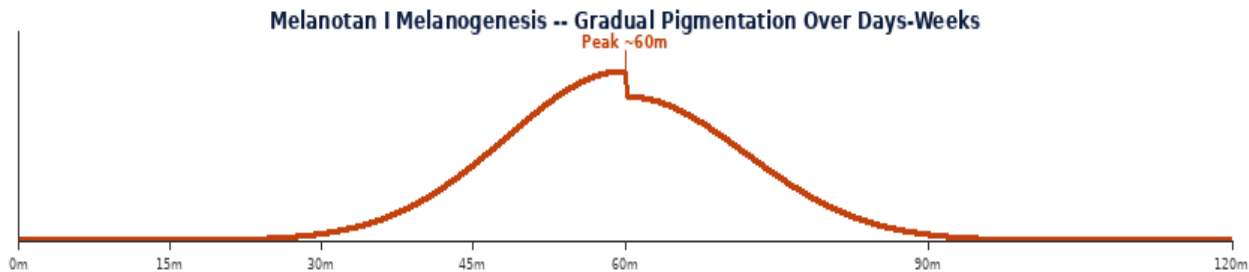
Melanotan I (Afamelanotide, CUV1647) is a linear analogue of alpha-Melanocyte Stimulating Hormone (alpha-MSH) with enhanced stability. It binds melanocortin receptors (MC1R) on melanocytes, stimulating eumelanin production independently of UV radiation. It is FDA-approved (as Scenesse) for erythropoietic protoporphyria (EPP). Research applications include photoprotection, melanogenesis, and skin pigmentation studies.

Melanotan I Vial Comparison



Parameter	Specification
Class	alpha-MSH analogue -- MC1R agonist (linear, more stable than alpha-MSH)
Vial Size	10mg x 10 vials Purity 99.8%
Reconstitution	1 mL BAC Water -> 10mg/mL (10,000 mcg/mL)
Standard Dose	250-1,000 mcg (0.25-1mg) per injection SubQ
Frequency	Every 1-3 days SubQ no fasting required
vs Melanotan II	Linear structure (not cyclic); no aphrodisiac effect; no appetite suppression; MC1R selective
Cycle & Storage	4-12 weeks 2-8 deg C use within 28 days

MECHANISM OF ACTION & RESEARCH BENEFITS



Pathway	Mechanism	Benefit
MC1R Activation	Binds melanocortin-1 receptor (MC1R) on melanocytes with high affinity; activates adenylate cyclase/cAMP pathway; upregulates tyrosinase; stimulates melanin synthesis; shifts towards eumelanin (darker, UV-protective pigment)	UV-independent melanogenesis; photoprotection research; skin darkening without UV exposure
Photoprotection	Eumelanin provides superior UV photoprotection vs pheomelanin; FDA-approved indication (EPP): protects against light-induced pain; reduces erythema and photosensitivity; potential sunburn protection in research	UV protection; erythropoietic protoporphyria; photosensitivity research
Selectivity vs Melanotan II	Linear (not cyclic) structure; high MC1R selectivity; does NOT significantly activate MC3R/MC4R (appetite, libido); no aphrodisiac or appetite-suppressing side effects; more selective photoprotection research tool	Cleaner melanogenic research; no sexual or appetite side effects

Benefit	Context
FDA-Approved Mechanism	Afamelanotide (Scenesse) is FDA-approved for EPP, providing the strongest clinical validation of MC1R-mediated photoprotection.
UV-Independent Tanning	Stimulates eumelanin without UV, offering a controlled research model for melanogenesis independent of sun exposure.
Cleaner Profile vs MT-II	No aphrodisiac activity, no appetite suppression -- MC1R selective. Preferred for pure melanogenesis research.

RECONSTITUTION & DOSE CALCULATION

All vials use 1 mL BAC Water. Room temperature. Swirl gently -- never shake.



1 mL BAC Water per vial. Roll gently -- NEVER shake.

Step	Action	Detail
1	Gather	Vial, BAC Water, insulin syringe (29-31G), swabs, sharps.
2	Clean stoppers	Wipe both stoppers. Air-dry 10-15 sec.
3	Draw 1mL BAC	Exactly 1 mL. Expel air bubbles.
4	Inject into vial	Aim at GLASS WALL. Slowly. Never onto powder.
5	Swirl & label	Roll 20-30 sec. Clear within 60 sec. Never shake. Label date. 2-8 deg C. Use within 28 days.



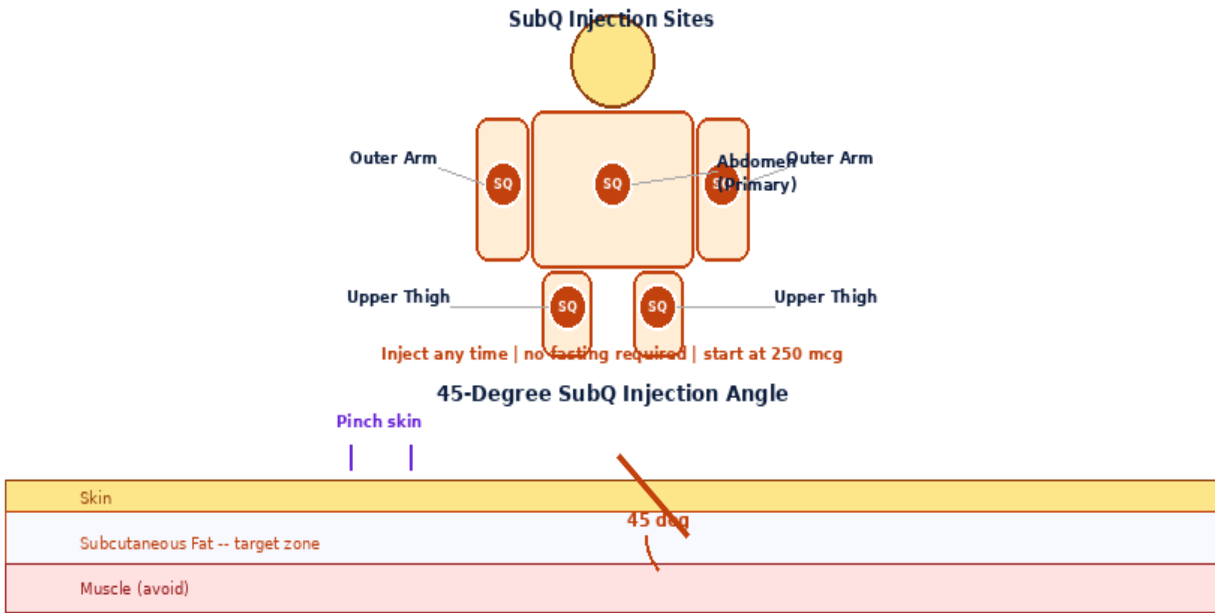
1mL Insulin Syringe | 29-31G | SubQ

Formula: $\text{Vol(mL)} = \text{Dose(mcg)} / \text{Conc(mcg/mL)}$ | **Units:** $\text{Vol} \times 100$

10mg x 10 -- 1mL BAC -> 10.0mg/mL (10,000mcg/mL)

Dose(mcg)	250mcg	500mcg	750mcg	1000mcg
Vol(mL)	0.0250	0.0500	0.0750	0.1000
Units	2.5U	5.0U	7.5U	10.0U
Doses/Vial	40	20	13	10

INJECTION TECHNIQUE, PROTOCOLS & GOLDEN RULES



8-Point Rotation Map -- Rotate Every Injection



Step	Action	Detail
1	Wash & prepare	Wash hands. 29-31G insulin syringe. No fasting needed.
2	Draw dose	Draw exact volume per dose table.
3	Inject SubQ	Abdomen preferred. Pinch 2-3cm. 45 deg. Rotate sites.
4	Rotate & dispose	8-point rotation. Press 5 sec. Sharps container.

Protocol	Dose	Frequency	Duration	Notes
Photoprotection Research	250-500 mcg	Every 1-2 days	4-12 wk	Lower dose. No UV needed for pigmentation.
Melanogenesis Research	750-1,000 mcg	Every 2-3 days	4-12 wk	Higher dose for enhanced pigmentation studies.

Side Effect	Likelihood	Management
Nausea (transient)	Common	Dose-related. Start low and escalate. Usually resolves.
Facial flushing	Common	Vasodilation. Mild. Transient.
New / changed moles	Monitor	Melanocyte stimulation. Regular dermatological monitoring required.
Fatigue (initial)	Occasional	Dose-related. Reduce dose if persistent.

Contraindications: Cancer history, pregnancy, under 18.

1. 1 mL BAC Water -> 10mg/mL (10,000 mcg/mL). Start at 250 mcg and escalate.
2. No fasting required. Inject any time of day.
3. Start low (250 mcg) to assess nausea tolerance before escalating.
4. Nausea is the most common side effect -- dose-related and usually transient.
5. MANDATORY: Regular dermatological monitoring for mole changes during cycle.
6. Contraindicated in personal or family history of melanoma or dysplastic nevi.
7. Rotate SubQ injection sites through 8-point abdomen map.
8. Store 2-8 deg C. Use within 28 days. Never freeze reconstituted peptide.
9. No aphrodisiac or appetite effect -- MT-I is MC1R selective (unlike Melanotan II).
10. Not for use during pregnancy, lactation, or in individuals under 18.

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